

lenge now is to ensure they become the building blocks of a health system that is equitable, efficient and centred on people's needs. Continuum of care and good referral mechanisms are important, with most health problems addressed at the primary and secondary levels, rather than in tertiary hospitals.

Much of the discussion around UHC tends to focus on health insurance. While insurance is essential, it is only one element of a well-functioning health system. The true measure of UHC is whether people can access the care they need, when they need it, without financial hardship. This includes preventive services, early diagnosis, management of chronic diseases, essential medicines, rehabilitation and palliative care, in addition to hospital treatment. Currently, out-of-pocket health expenditure still accounts for about 40% of health spending in India.

No country offers a perfect model in this regard. The United Kingdom relies largely on a tax-funded National Health Service, while countries such as Germany and France have social health insurance systems. The United States spends considerably more on healthcare than any other country (17% of GDP) yet continues to grapple with inequities in access and affordability. The lesson from countries that have achieved universal health coverage is that, despite differences in financing, they have invested consistently in strong primary healthcare, effective regulation, public health and accountable institutions.

LEARNING FROM OTHERS

Many of these high-income countries spend considerably more on healthcare than India. Yet their greatest strength lies not simply in higher expenditure, but in how they organise care by placing primary healthcare at the centre, regulating providers effectively

and ensuring continuity across the health system. These are lessons India can adapt, even within its own resource constraints.

India's path, however, will necessarily be shaped by the unique characteristics of its own health system. One of its defining features is the significant role of the private sector, which provides most outpatient care and a substantial proportion of hospital services. Rather than viewing public and private healthcare as competing models, the focus should be on

hospitals. The initiative illustrates how clearly defined protocols, accountability mechanisms and effective coordination can harness private sector capacity in the service of public health. As India expands partnerships with private providers, similar approaches can help improve access while maintaining quality and accountability.

Equally important is strengthening primary healthcare, which is the foundation of the health system. Hospital-based insurance cannot substitute for accessible, high-



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how both can contribute to better health outcomes. This places a premium on the government's role as the steward of the health system: setting standards, ensuring quality, regulating where necessary, ensuring transparency and creating incentives that reward good outcomes rather than volume of care.

The Clinical Establishment Act is still poorly implemented in most states. Well-designed public-private partnerships demonstrate what this can achieve. Tamil Nadu's Nammai Kaakkum 48 initiative under the Chief Minister's Comprehensive Health Insurance Scheme ensures that people injured in road traffic accidents receive timely emergency care through a network of empanelled public and private

quality primary care and support the long-term management of chronic conditions. India's investment in health and wellness Centres is therefore one of its most important health reforms. Their success, however, will depend on empowered frontline teams comprising Community Health Officers, nurses and ASHAs, supported through continuous training, mentoring and digital tools that enhance care rather than add to administrative burdens. The team approach is important, with the PHC medical officer and the entire team under him/her responsible for the health of the population under that PHC. Incentives can be provided for teams at block or district level that achieve the best population health outcomes.